

Policy on Patient Safety Programme

«Hospital Name»

Document control

Document No.	HCO/PSQ/POL/01	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Patient Safety Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» implements a structured patient safety programme.

The patient safety programme is developed, implemented and maintained by a multi-disciplinary safety committee. The programme covers the major elements related to patient safety and incidents ranging from “no harm” to “sentinel events”. Designated patient safety officer(s) coordinate implementation of the patient safety programme. The organisation performs proactive analysis of patient safety risks and makes improvements accordingly. The programme is reviewed and updated at least once a year and the organisation adapts and implements national and international patient-safety goals, solutions or frameworks.

1. Purpose

The purpose of this policy is to establish the structure and approach for the patient safety programme, including the multi-disciplinary safety committee, patient safety officer, programme scope and coverage, proactive risk analysis, annual programme review and adoption of national and international patient-safety goals, solutions or frameworks.

This policy does not cover matters outside the patient safety programme.

2. Scope

This policy applies across «Hospital Name» to the patient safety programme and the activities supporting patient safety and quality improvement.

It applies to the multi-disciplinary safety committee, designated patient safety officer(s), leadership, relevant staff, departments and, where applicable, the governing body.

3. Policy standards

3.1 Patient safety programme and safety committee

The patient safety programme shall be developed, implemented and maintained by a multi-disciplinary safety committee. The safety committee shall include representatives from administration, facility management, safety officers, clinicians, nurses, paramedical staff and various clinical and support departments.

The safety committee meets every month to co-ordinate development, implementation and monitoring of safety plans and policies. The patient safety programme shall be documented as a manual incorporating all the requirements of this standard.

The committee shall ensure that patients are protected from harm either from the environment or from lack of appropriate care or lack of safety measures.

3.2 Programme comprehensiveness

The patient safety programme shall be comprehensive and shall address all elements of safety for clinical and support services.

3.3 Incident coverage

The programme covers incidents ranging from 'no harm' to 'sentinel events'.

The organisation shall clearly define what constitutes 'no harm' and 'sentinel events' for the purposes of the patient safety programme.

3.4 Patient safety officer

Designated patient safety officer(s) shall co-ordinate implementation of the patient safety programme. The patient safety officer(s) shall have good knowledge of patient and general safety.

The patient safety officer shall be a doctor or nurse with experience in the application of risk management to clinical domains. The role and responsibilities of the patient safety officer(s) shall be defined and documented. The patient safety officer(s) shall report to top management.

Patient safety champions are identified and developed across the organisation and are supported to drive patient safety.

3.5 Proactive risk analysis

The organisation performs proactive analysis of patient safety risks and makes improvements accordingly. At a minimum, one patient safety related risk shall undergo proactive analysis every year.

The safety committee is responsible for proactive risk assessment, covering both clinical and non-clinical processes and areas. Proactive analysis may be done using tools such as Hazard Identification and Risk Analysis (HIRA), Failure Modes and Effects Analysis (FMEA), Hazard Vulnerability Analysis (HVA), fault tree analysis or simulations.

Improvements are made based on the findings of the proactive analysis to eliminate unsafe actions and conditions that could lead to harmful incidents.

3.6 Annual programme review

The patient safety programme shall be reviewed and updated at least once a year. The review shall be done by the safety committee at pre-defined intervals as defined by the organisation in the safety manual but at least once a year.

The update shall be based on newer literature on patient safety and the findings of reviews.

The review shall at a minimum include reports of facility inspection rounds, patient safety incidents, risk management and analysis of key safety indicators.

The minutes of the review meetings shall be recorded and maintained. If the annual review does not identify any opportunities for improvement, this shall be documented in the minutes of the safety committee meeting.

3.7 National and international patient-safety goals

The organisation adapts and implements national and international patient-safety goals, solutions or frameworks.

At a minimum, the organisation shall adhere to the current National Patient-Safety Framework, WHO Patient Safety Solutions and the International Patient Safety Goals.

4. Non-negotiable rules

1. Do not operate the patient safety programme without a multi-disciplinary safety committee that includes representatives from administration, facility management, safety officers, clinicians, nurses and paramedical staff.
2. Do not allow the safety committee to meet less frequently than monthly.
3. Do not leave the patient safety programme undocumented; it shall be documented as a manual.
4. Do not leave the definitions of 'no harm' and 'sentinel events' unspecified for the purposes of the patient safety programme.
5. Do not leave a patient safety officer(s) undesignated; the role and responsibilities of the patient safety officer(s) shall be defined and documented.
6. Do not complete fewer than one patient safety risk proactive analysis per year.
7. Do not allow the patient safety programme to go unreviewed for more than a year; the safety committee shall conduct and record the review.
8. Do not fail to adapt and implement national and international patient-safety goals, solutions or frameworks.

5. What we do

5.1 Implement the patient safety programme and safety committee

The patient safety programme is developed, implemented and maintained by a multi-disciplinary safety committee. The safety committee includes representatives from administration, facility management, safety officers, clinicians, nurses, paramedical staff and various clinical and support departments.

The safety committee meets every month to co-ordinate development, implementation and monitoring of safety plans and policies.

The safety committee ensures that patients are protected from harm either from the environment or from lack of appropriate care or lack of safety measures.

The patient safety programme is documented as a manual.

5.2 Maintain programme comprehensiveness

The patient safety programme is comprehensive and addresses all elements of safety for clinical and support services.

5.3 Cover the full incident range

The programme covers incidents ranging from no harm to sentinel events.

The definitions of no harm and sentinel events are documented for the purposes of the patient safety programme.

5.4 Designate the patient safety officer

Designated patient safety officer(s) co-ordinate implementation of the patient safety programme. The patient safety officer(s) has good knowledge of patient and general safety and experience in the application of risk management to clinical domains.

The role and responsibilities are defined and documented. The patient safety officer(s) reports to top management.

Patient safety champions are identified and developed across the organisation and supported to drive patient safety.

5.5 Perform proactive risk analysis

The organisation performs proactive analysis of patient safety risks and makes improvements accordingly. At a minimum, one patient safety related risk undergoes proactive analysis every year.

The safety committee is responsible for proactive risk assessment, covering both clinical and non-clinical processes and areas. Tools such as HIRA, FMEA, HVA, fault tree analysis or simulations may be used. Improvements are made based on the findings to eliminate unsafe actions and conditions.

5.6 Review the programme annually

The patient safety programme is reviewed and updated at least once a year by the safety committee.

The update is based on newer literature on patient safety and the findings of reviews.

The review includes at a minimum reports of facility inspection rounds, patient safety incidents, risk management and analysis of key safety indicators.

The minutes of the review meetings are recorded and maintained. When the annual review does not identify any opportunities for improvement, this is documented in the minutes.

5.7 Implement national and international patient-safety goals

The organisation adapts and implements national and international patient-safety goals, solutions or frameworks.

At a minimum, the National Patient-Safety Framework, WHO Patient Safety Solutions and the International Patient Safety Goals are adhered to.

6. Stop-work authority

6.1 Patient safety committee trigger

Absence of a functioning multi-disciplinary safety committee developing, implementing and maintaining the patient safety programme is CORE non-compliance. The patient safety programme must be developed, implemented and maintained by a multi-disciplinary safety committee that meets every month. Failure to have such a committee constitutes the specified stop-work trigger.

6.2 National and international patient-safety goals trigger

Absence of adoption and implementation of national and international patient-safety goals, solutions or frameworks is CORE non-compliance. The organisation must at a minimum adhere to the current National Patient-Safety Framework, WHO Patient Safety Solutions and the International Patient Safety Goals. Failure to have adopted and implemented these constitutes the specified stop-work trigger.

7. Governance and responsibility

7.1 Leadership

Leadership supports the patient safety programme and the multi-disciplinary safety committee in developing, implementing and maintaining the programme.

Leadership supports the designation of patient safety officer(s) and ensures the role and responsibilities are defined and documented.

7.2 Multi-disciplinary safety committee

The multi-disciplinary safety committee develops, implements and maintains the patient safety programme. The safety committee meets every month to co-ordinate development, implementation and monitoring of safety plans and policies.

The safety committee is responsible for the proactive risk assessment activities and for the annual programme review.

7.3 Patient safety officer(s)

Designated patient safety officer(s) co-ordinate implementation of the patient safety programme and report to top management.

7.4 Relevant staff and departments

Relevant staff and departments participate in the patient safety programme activities.

Patient safety champions across the organisation support implementation of the patient safety programme.

7.5 Governing body

Where applicable, the governing body is informed of the patient safety programme and its outcomes.

8. Quality monitoring

Monitoring area	What is monitored
Safety committee	Multi-disciplinary committee composition including required representatives, and monthly meetings.
Patient safety programme	Comprehensiveness of the programme covering all elements of safety for clinical and support services, and documentation as a manual.
Incident coverage	Coverage of incidents from no harm to sentinel events and documented definitions of 'no harm' and 'sentinel events'.
Patient safety officer	Designation of patient safety officer(s), documented role and responsibilities, and reporting line to top management.
Proactive risk analysis	At least one patient safety related risk proactively analysed per year and improvements made based on findings.
Programme review	Annual safety committee review including facility inspection round reports, patient safety incidents, risk management and key safety indicator analysis, with recorded minutes.
National and international patient-safety	Adoption and implementation of national

goals	and international patient-safety goals, solutions or frameworks.
-------	--

Quality monitoring shall distinguish between data collection and documented improvement response. Data collection without a documented improvement response does not meet the stated requirement.

9. Training and staff acknowledgement

Staff involved in the patient safety programme shall be familiar with the processes described in this policy according to their responsibilities.

Relevant staff participate in activities related to the patient safety programme according to their responsibilities.

Staff acknowledgement:

I have read the Policy on Patient Safety Programme of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to relevant staff and departments involved in the patient safety programme.

11. Abbreviations

Abbreviation	Meaning
NABH	National Accreditation Board for Hospitals and Healthcare Providers
PDSA	Plan-Do-Study-Act
PSQ	Patient Safety and Quality Improvement
FMEA	Failure Modes and Effects Analysis
HIRA	Hazard Identification and Risk Analysis
HVA	Hazard Vulnerability Analysis

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
PSQ.1.a	CORE*	Sections 3.1, 5.1 and 6 address

		the development, implementation and maintenance of the patient safety programme by the multi-disciplinary safety committee, the monthly meeting requirement, documentation as a manual, and the specified CORE stop-work trigger.
PSQ.1.b	Commitment	Sections 3.2 and 5.2 address the comprehensiveness of the patient safety programme and its coverage of all elements of safety for clinical and support services.
PSQ.1.c	Commitment	Sections 3.3 and 5.3 address the programme's coverage of incidents ranging from no harm to sentinel events and the requirement to clearly define both 'no harm' and 'sentinel events'.
PSQ.1.d	Commitment	Sections 3.4 and 5.4 address the designation of a patient safety officer(s), the knowledge and background requirements, the definition and documentation of the role and responsibilities, reporting to top management, and the identification of patient safety champions.
PSQ.1.e	Commitment	Sections 3.5 and 5.5 address proactive analysis of patient safety risks, the minimum frequency of one risk analysis per year, and the making of improvements based on findings.
PSQ.1.f	Commitment	Sections 3.6 and 5.6 address the annual review and update of the patient safety programme by the safety committee, the minimum scope of the review, recording and maintenance of minutes, and documentation when no

		improvement opportunities are identified.
PSQ.1.g	CORE	Sections 3.7, 5.7 and 6 address the adoption and implementation of national and international patient-safety goals, solutions or frameworks, including the National Patient-Safety Framework, WHO Patient Safety Solutions and the International Patient Safety Goals, and the specified CORE stop-work trigger.

13. Required Records/Evidence Checklist

Patient safety programme and safety committee

- Patient safety programme documented as a manual covering all requirements of this standard.
- Multi-disciplinary safety committee membership records showing required representatives from administration, facility management, safety officers, clinicians, nurses and paramedical staff.
- Safety committee meeting minutes showing meetings held every month.
- Evidence that the patient safety programme is developed, implemented and maintained by the safety committee.

Programme scope and incident coverage

- Documentation showing the patient safety programme is comprehensive and covers all elements of safety for clinical and support services.
- Documented definitions of 'no harm' and 'sentinel events' for the organisation.

Patient safety officer

- Appointment letter or designation record for the patient safety officer(s).
- Documentation of the role and responsibilities of the patient safety officer(s).
- Record showing the patient safety officer(s) reports to top management.
- Records of patient safety champions identified and developed across the organisation.

Proactive risk analysis

- Records of at least one proactive risk analysis conducted per year.
- Risk analysis records documenting the tool used (for example HIRA, FMEA, HVA or similar) and findings.
- Documented improvements made based on the findings of the proactive risk analysis.

Annual programme review

- Records of the annual patient safety programme review conducted by the safety committee.
- Review scope documentation showing inclusion of facility inspection round reports, patient safety incidents, risk management and key safety indicator analysis.
- Minutes of the programme review meetings.
- Where no improvement opportunities were identified in the annual review, documentation of this in the meeting minutes.

National and international patient-safety goals

- Records showing adoption and implementation of the National Patient-Safety Framework, WHO Patient Safety Solutions and the International Patient Safety Goals, or equivalent national and international patient-safety goals adopted by the organisation.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.1, “The organisation implements a structured patient safety programme.”
- National Accreditation Board for Hospitals and Healthcare Providers. Guidebook to NABH Accreditation Standards for Hospitals, 6th Edition. Glossary of Terms. Definitions of safety programme, no harm and sentinel events.
- Ministry of Health and Family Welfare, Government of India. (2018). https://mohfw.gov.in/sites/default/files/national%20patient%20safety%20implimentation_for%20web.pdf National Patient Safety Implementation Framework (2018-2025).
- World Health Organization. (n.d.). <https://www.who.int/teams/integrated-health-services/patient-safety/research/patient-safety-solutions> Patient Safety Solutions.
- Joint Commission International. (n.d.). <https://www.jointcommissioninternational.org/standards/international-patient-safety-goals> International Patient Safety Goals.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.